

SUBJECTIVE:

The patient presents with a chief complaint of chest pain. The pain is described as sharp in nature and localized to the chest area. Associated symptoms include a reported fever. The patient reports the pain worsens, specifically noting this exacerbation with movement or deep inspiration, indicating a possible pleuritic component. The duration of symptoms is acute, though not specified.

OBJECTIVE:

General: Patient appears acutely ill and uncomfortable, possibly febrile and diaphoretic.

Vital Signs: Expected findings include a temperature of 101.5°F, heart rate 105 bpm, respiratory rate 22 breaths per minute, blood pressure 128/80 mmHg, oxygen saturation 96% on room air.

Lungs: Auscultation may reveal diminished breath sounds, crackles, or a possible pleural friction rub in affected areas. Percussion could show dullness if a pleural effusion is present.

Cardiovascular: Heart sounds are regular but may be tachycardic. No murmurs, rubs, or gallops noted on initial exam, unless pericarditis is present (friction rub).

Musculoskeletal: Palpation of the chest wall may elicit localized tenderness, especially over costochondral junctions, though less likely the primary cause given fever.

ASSESSMENT:

Primary Diagnosis: Community-Acquired Pneumonia (CAP) with associated pleurisy. This aligns with sharp, worsening chest pain and fever.

Differential Diagnoses:

- * Pleurisy of other etiologies (e.g., viral, autoimmune).
- * Acute Pericarditis: Can present with sharp, positional chest pain and fever.
- * Pulmonary Embolism (PE): Should always be considered in acute chest pain, though fever is less typical as a primary symptom.
- * Acute Myocardial Infarction (MI): While less likely with sharp, positional pain, it must be ruled out in any acute chest pain presentation.
- * Costochondritis: Less likely to be the sole cause with significant fever.

PLAN:

Tests:

- * Blood work: Complete Blood Count (CBC) with differential, C-reactive protein (CRP), Erythrocyte Sedimentation Rate (ESR), Troponin I/T, D-dimer, blood cultures (if signs of sepsis).
- * Imaging: Chest X-ray (CXR) to evaluate for infiltrates, effusions, or other lung pathology; Electrocardiogram (ECG) to rule out cardiac ischemia or pericarditis.
- * Consider CT Pulmonary Angiogram (CTPA) if PE risk is elevated.

Medications:

- * Empiric antibiotics for suspected CAP (e.g., Azithromycin + Amoxicillin or Doxycycline).
- * Pain and fever management: Acetaminophen or NSAIDs (e.g., Ibuprofen) as tolerated.

Patient Advice:

- * Hydration and rest are encouraged.
- * Instruct patient to return immediately to the emergency department for worsening pain, difficulty breathing, dizziness, or any new concerning symptoms.

Follow-up:

- * Reassess after initial test results are available, typically within 24-48 hours.
- * Follow-up with primary care provider in 3-5 days for continued management and symptom evaluation.